

## HEALTHCARE INTERNATIONALIZATION : HOSPITALS ACQUISITIONS BETWEEN 2010 AND 2025 IN BRAZIL

José Lener de Souza Cárdenas (UEM), E-mail: [lenercardenas17@gmail.com](mailto:lenercardenas17@gmail.com)

Fabiane Cortez Verdu (UEM), E-mail: [fcoverdu@uem.br](mailto:fcoverdu@uem.br)

Maurício Reinert do Nascimento (UEM), E-mail: [mrnascimento@uem.br](mailto:mrnascimento@uem.br)

### ABSTRACT

The aim of this study is to map out foreign acquisitions of hospitals in Brazil between 2015 and 2025 following the regulatory opening of Federal Law No. 13,097/2015. Through an exploratory qualitative analysis of secondary data, 9 cross-border transactions were cataloged and examined. The findings reveal a strong initial concentration, with 78% of deals occurring in 2015–2016 immediately after the legal reform. Two distinct entry strategies emerged: direct operational consolidation led by UnitedHealth Group/Amil (67% of cases) and minority equity investments by financial funds (GIC, Carlyle, CVC). US investors predominated in the period. The study concludes that foreign entry combined operational expansion with selective financial positioning, underscoring the role of regulatory stability in attracting foreign direct investment.

**Key words:** Healthcare Internationalization; Foreign Direct Investment; Hospital Acquisitions; Mergers and Acquisitions; Brazil.

### RESUMO

O objetivo deste estudo é mapear as aquisições estrangeiras de hospitais no Brasil entre 2015 e 2025, após a abertura regulatória promovida pela Lei Federal nº 13.097/2015. Por meio de uma análise qualitativa exploratória de dados secundários, foram catalogadas e examinadas 9 transações transfronteiriças. Os resultados revelam uma forte concentração inicial, com 78% dos negócios ocorrendo em 2015–2016, imediatamente após a reforma legal. Duas estratégias distintas de entrada emergiram: a consolidação operacional direta liderada pelo UnitedHealth Group/Amil (67% dos casos) e aportes financeiros via participações acionárias minoritárias por fundos financeiros (GIC, Carlyle, CVC). Investidores norte-americanos predominaram no período. O estudo conclui que a entrada do capital estrangeiro articulou expansão operacional com posicionamento financeiro seletivo, destacando o papel da estabilidade regulatória na atração de investimentos diretos estrangeiros.

**Palavras chave:** Internacionalização da saúde; Investimento direto estrangeiro; Aquisição de hospitais; Fusões e aquisições; Brasil.

### RESUMEN

El objetivo de este estudio es mapear las adquisiciones extranjeras de hospitales en Brasil entre 2015 y 2025 tras la apertura regulatoria promovida por la Ley Federal N.º 13.097/2015.

A través de un análisis cualitativo exploratorio de datos secundarios, se catalogaron y examinaron 9 transacciones transfronterizas. Los hallazgos revelan una fuerte concentración inicial, con el 78% de los acuerdos ocurridos en 2015–2016, inmediatamente después de la reforma legal. Surgieron dos estrategias de entrada distintas: la consolidación operacional directa liderada por UnitedHealth Group/Amil (67% de los casos) y las inversiones financieras en participaciones minoritarias por parte de fondos de inversión (GIC, Carlyle, CVC). Los inversionistas estadounidenses predominaron en el período. El estudio concluye que la entrada de capital extranjero combinó la expansión operacional con un posicionamiento financiero selectivo, destacando el papel de la estabilidad regulatoria en la atracción de inversión extranjera directa.

**Palabras clave:** Internacionalización de la salud; Inversión extranjera directa; Adquisición de hospitales; Fusiones y adquisiciones; Brasil.

## 1 INTRODUCTION

The internationalization of healthcare markets has intensified over the past decade, driven by demographic pressures, rising demand for specialized medical services, and the search for operational efficiency across global health systems. Brazil offers a distinctive setting for examining this phenomenon: it combines a constitutionally guaranteed universal public health system with an expanding private healthcare sector (Paim, Travassos, Almeida, Bahia, & Macinko, 2011), and it experienced a significant regulatory shift in 2015, when Federal Law No. 13,097/2015 lifted longstanding restrictions on foreign capital participation in the healthcare sector (Scheffer, 2022).

Despite the relevance of this regulatory change, empirical evidence on how foreign investors have actually engaged with the Brazilian hospital sector in its aftermath remains fragmented, often limited to isolated case reports in the business press rather than systematic mapping. Cross-border mergers and acquisitions have been identified in the literature as one of the principal mechanisms through which foreign capital enters healthcare markets, providing rapid access to infrastructure, licensed operations, and existing patient networks (Dunning, 2001; Burns, Wholey, & Kralovec, 2015; Postma & Roos, 2016); however, systematic empirical accounts of this process in the Brazilian context remain scarce. Understanding the scale, nature, and evolution of these transactions is important not only for academic discussions on foreign direct investment (FDI) and market entry strategies, but also for policymakers assessing the effects of regulatory liberalization on a sensitive sector directly tied to public welfare. Against this background, the aim of this study is to map out foreign acquisitions of hospitals in Brazil between 2015 and 2025, identifying patterns related to the

timing, investor profile, geographic origin, and type of transaction involved. To this end, the article first reviews the theoretical literature on internationalization strategies and the specific characteristics of the Brazilian healthcare system; it then describes the methodological procedures adopted for data collection and analysis; presents the empirical findings and discusses them in light of the theoretical framework; and, finally, offers concluding remarks on the study's contributions and limitations.

## **2 THEORETICAL APPROACH**

This section aims to expose the theoretical framework that will guide this study, at first a brief discussion about the internationalization process, followed by a topic about internationalization and healthcare and then the context of the Unified Healthcare System (SUS) and foreign capital.

### **2.1 THE INTERNATIONALIZATION PROCESS**

By opting for a form of internationalization, organizations must seek their main motivation for pursuing business abroad, where some will be reactive and some proactive (Cavusgil; Knight; Riesenberger, 2010). For companies that embark on initiatives such as exporting, licensing, or franchising, the reasons for internationalization tend to be relatively straightforward. In most cases, the goal is to maximize returns on investments made in goods, services, and know-how by seeking a broader base of consumers located abroad (Cavusgil; Knight; Riesenberger, 2010).

Push and pull factors act as initial triggers for internationalization, combining unfavorable conditions in the domestic market, such as declining demand and increased competition, with attractive opportunities abroad, such as faster growth and lower competition. Internationalization can also occur accidentally or unintentionally, when unexpected events, such as business contacts or commercial opportunities, lead a company to enter international markets (Cavusgil; Knight; Riesenberger, 2010).

Johanson and Vahlne (2009) proposed in their highly recognized article “The Uppsala internationalization process model revisited: From liability of Foreignness to Liability of Outsidership” revisiting their original 1977 model proposal emphasized that internationalization as a gradual and dynamic process rather than a fully planned decision based on optimal market-entry strategies.

Based on observations of Swedish firms, Johanson and Vahlne found that companies often began with occasional exporting, then progressively increased their commitment through intermediaries, sales subsidiaries, and eventually foreign production. Firms also tended to enter geographically and culturally closer markets before moving into more psychologically distant ones. The model emphasizes uncertainty and bounded rationality, arguing that firms learn from experience, which increases their market knowledge and influences subsequent commitment decisions. Thus, internationalization develops incrementally through the interaction between market knowledge, market commitment, current activities, and commitment decisions, as long as market performance and prospects remain favorable (Johanson; Vahlne, 2009).

Followed by their revision of the original model, they explain firm internationalization from the perspective of business networks. In this model, the firm is embedded in a network of actors and interdependent relationships that can simultaneously enable and constrain its actions. Therefore, internationalization should not be understood only as entering a new foreign market, but also as a process of strengthening the firm’s position within these networks. Existing business relationships are particularly important because they allow firms to identify and exploit opportunities, influencing both the geographical market they choose to enter and the mode of entry they adopt (Johanson; Vahlne, 2009).

Another central aspect of the revised model is the importance of knowledge, learning, trust, and commitment within the relationships established by the firm. According to Johanson and Vahlne (2009), knowledge acquired through relationships and experience contributes to the identification and exploitation of opportunities. Thus, internationalization is less concerned with simply reducing uncertainty about foreign markets and more with developing

opportunities through existing relationships. Commitment to partners can also provide firms with access to knowledge that may be restricted to members of a particular network, facilitating the discovery or creation of new international opportunities.

Therefore, the internationalization process should be understood as an ongoing process in which firms progressively adapt their strategies as they gain experience, knowledge, and access to new opportunities. Rather than representing a single decision, internationalization involves a continuous process of learning, adaptation, and increasing commitment to foreign markets.

## 2.2 INTERNATIONALIZATION AND HEALTHCARE

Firms can internationalize through different foreign market entry strategies, which vary according to the level of resource commitment, desired control, and risk exposure. The main entry modes include exporting, licensing, franchising, strategic alliances, joint ventures, and Foreign Direct Investment (FDI), with FDI representing the mode that requires the highest level of resource commitment and managerial control (Cavusgil, Knight, & Riesenberger, 2010; Kotabe & Helsen, 2017). According to the stage model of internationalization, firms typically begin their international expansion through entry modes that involve lower levels of investment and risk, such as exporting, and gradually progress toward more complex forms as they accumulate knowledge and experience in foreign markets (Cavusgil, Knight, & Riesenberger, 2010).

In this process, FDI represents the most advanced stage of internationalization and may occur through greenfield investments, involving the establishment of new operations, or through cross-border mergers and acquisitions, which provide rapid access to infrastructure, strategic assets, customers, and market share (Kotabe & Helsen, 2017).

The evolution of foreign direct investment (FDI) has transformed the way firms expand internationally, particularly through the growing importance of cross-border mergers and acquisitions. Revisiting the Eclectic Paradigm, Dunning (2001) argues that international production is increasingly shaped not only by the exploitation of existing competitive advantages but also by firm's efforts to acquire strategic assets, access new markets, and

strengthen their long-term competitiveness. Foreign capital refers to financial resources owned by individuals or firms based abroad that are invested in the economy of another country through mechanisms such as equity acquisitions, loans, financing arrangements, and asset transactions (OECD, 2026).

In this context, the attractiveness of host countries depends on a combination of locational characteristics, including institutional quality, regulatory frameworks, market conditions, infrastructure, and the availability of valuable assets. These changes have reinforced the role of mergers and acquisitions as a dominant mode of international expansion, particularly in sectors where market entry through greenfield investments is constrained by regulatory, organizational, or structural factors (Dunning, 2001).

According to the Eclectic Paradigm, foreign direct investment (FDI) is influenced by the interaction of ownership, location, and internalization advantages. In the healthcare sector, universal healthcare systems can increase market attractiveness by expanding demand for health services, while favorable institutional conditions, qualified healthcare professionals, and well-developed infrastructure strengthen location-specific advantages. In addition, stable regulatory environments reduce transaction costs and uncertainty, making these markets more attractive for foreign investment (Ouakil; Sarsar; Soussane, 2025). However the internationalization of healthcare has occurred later than in most other sectors of the economy, lagging behind both manufacturing and many service industries (Shenkar; Liang, Shankar, 2021).

Overall, Ouakil, Sarsar and Soussane (2025) suggests that universal healthcare coverage enhances a country's attractiveness for FDI in the health sector by providing a stable institutional environment, expanding market opportunities, and reducing investment risks. These conditions can attract foreign investors while supporting the expansion of healthcare services and improving access to care.

Within this process, cross-border mergers and acquisitions have apparently emerged as one of the principal mechanisms through which foreign capital enters healthcare markets (Dunning, 2001). By acquiring existing healthcare providers, investors can rapidly establish a market presence, access operational infrastructure, and overcome barriers associated with entering

highly regulated sectors (Postma & Roos, 2016). Consequently, acquisitions have become an important feature of the internationalization of private healthcare systems, particularly in response to increasing competition and the search for greater efficiency and market consolidation (Burns; Wholey; Kralovec, 2015).

The characteristics of healthcare services make hospitals particularly attractive targets for foreign direct investment through mergers and acquisitions. Unlike greenfield investments, hospital acquisitions provide immediate access to established facilities, licensed operations, specialized human resources, and existing patient networks, thereby reducing entry barriers in highly regulated healthcare markets. As a result, cross-border acquisitions have become one of the main channels through which foreign capital expands within healthcare systems, especially in countries where private healthcare provision represents an important component of service delivery (Burns, Wholey, & Kralovec, 2015; Postma & Roos, 2016).

Therefore, the internationalization of the healthcare sector, particularly through foreign direct investment in hospital acquisitions, has become an increasingly relevant topic of investigation. In this context, Brazil provides an important setting for examining how institutional arrangements and regulatory changes have shaped the participation of foreign capital in the healthcare market.

### 2.3 THE BRAZILIAN HEALTHCARE SYSTEM AND FOREIGN CAPITAL

The establishment of the Unified Health System (SUS) following the enactment of the 1988 Federal Constitution guaranteed universal access to healthcare in Brazil. With the promulgation of the Constitution, health was recognized as a fundamental right of all Brazilian citizens and a duty of the State (Brasil, 2026). Within this context, the Unified Health System (SUS) was formally established as the primary mechanism for promoting universality, comprehensiveness, and equity in access to healthcare services throughout the country (Peduzzi, 2016).

The Brazilian healthcare system is organized as a public–private structure comprising three subsectors: the public subsector, represented by the Unified Health System (SUS), which provides universal access and is financed by federal, state, and municipal resources; the private subsector, in which healthcare services are financed through public and/or private

resources; and the private health insurance subsector (Paim, Travassos, Almeida, Bahia, & Macinko, 2011).

Despite the universal coverage provided by the Unified Health System (SUS), Brazil has experienced a sustained increase in healthcare demand driven by population aging, the growing burden of chronic diseases, and technological advances in medical care. These trends have contributed to rising healthcare expenditures and intensified the participation of private providers in the delivery and financing of healthcare services. According to the OECD (2021), although Brazil allocates a relatively high share of its gross domestic product to health, a substantial proportion of total health expenditure is financed through private sources, including private health insurance and out-of-pocket payments, highlighting the significant role of the private sector within the Brazilian healthcare system.

Similarly, the World Health Organization and the World Bank (2026) have emphasized that demographic and epidemiological transitions, coupled with increasing healthcare spending, have stimulated investments aimed at expanding healthcare infrastructure and improving service capacity. In this context, the Brazilian healthcare market has become increasingly attractive to domestic and international investors, particularly in hospital services, where the search for economies of scale, operational efficiency, and market expansion has accelerated investment activity.

At the national level, the Organic Health Law (*Lei Orgânica da Saúde*) restricted the participation of foreign capital in the Brazilian healthcare system, while the 1988 Federal Constitution established that foreign capital should serve the national interest and prohibited its participation in healthcare, except when authorized by specific legislation (Scheffer, 2022). Between 2015 and 2021, Federal Law No. 13,097/2015 authorized unrestricted foreign investment in healthcare services in Brazil by amending the Organic Health Law. This change was introduced through an amendment to a provisional measure that had originally addressed tax-related issues, giving rise to criticism regarding the legislative process. Supporters argued that the reform would reduce financing asymmetries and enhance competitiveness within the private healthcare sector (Scheffer, 2022).

In light of these regulatory and theoretical dynamics, hospital acquisitions represent the concrete mechanism through which foreign direct investment materializes within the Brazilian healthcare market. The legislative opening enacted by Law No. 13,097/2015 effectively aligned Brazil with the global trends of sectoral internationalization and strategic-asset seeking behavior previously discussed (Dunning, 2001; Burns, Wholey, & Kralovec, 2015; Postma & Roos, 2016). Therefore, investigating hospital acquisitions between 2015 and 2025 serves as the logical bridge to operationalize the study of FDI inflows, establishing the empirical focus that grounds the methodological approach of this research.

### **3 METHODOLOGY**

This study adopts an exploratory and descriptive research design based on secondary data. The objective is to map out foreign acquisitions of hospitals in Brazil between 2015 and 2025.

Data were collected from publicly available secondary sources, primarily through news reports published by business, healthcare, and financial media. The search focused on identifying foreign acquisitions of Brazilian hospitals between 2015 and 2025 using predefined keywords related to mergers and acquisitions in the hospital sector. Whenever available, the information is reported in the following news articles: Folha de São Paulo, PPTA Saúde, ABIMED, CNN, UOL Economics, Reuters and NeoFeed.

The collected material was analyzed using qualitative content analysis. Each identified acquisition was coded according to predefined analytical categories, including the year of the transaction, acquiring company, target hospital or hospital group, country of origin of the foreign investor, location of the acquired hospital, and, when available, the announced transaction value. The classified data were then examined to identify patterns and trends in foreign acquisitions of hospitals in Brazil throughout the study period.

### **4 ANALYSIS AND RESULTS**

Based on the data collected from news reports and KPMG reports, it was possible to map the acquisition of hospitals by foreign entities in Brazil between 2015 and 2025. The following section presents the analysis and results based on the collected data.

#### 4.1 DATA ANALYSIS AND RESULTS

Initially, the collected data were organized by year, investor, country of origin, target hospital, type of transaction, transaction value (when disclosed), and source. These data are presented in **Table 1** below.

**Table 1** - Foreign Acquisitions in the Brazilian Hospital Sector (2015–2025).

| Year | Investor                      | Country       | Target                                               | Type of Transaction                        | Transaction Value | Documentary Source                   |
|------|-------------------------------|---------------|------------------------------------------------------|--------------------------------------------|-------------------|--------------------------------------|
| 2015 | UnitedHealth Group (via Amil) | United States | Hospital Samaritano (São Paulo)                      | Hospital acquisition                       | BRL 1.3 billion   | <i>Folha de S.Paulo</i>              |
| 2015 | GIC                           | Singapore     | Rede D'Or São Luiz                                   | Equity stake acquisition (16%)             | BRL 3.3 billion   | Reuters / UOL                        |
| 2015 | The Carlyle Group             | United States | Rede D'Or São Luiz                                   | Equity stake acquisition (8.3%)            | BRL 1.75 billion  | Bloomberg / UOL                      |
| 2016 | UnitedHealth Group (via Amil) | United States | Hospital Santa Helena (São Paulo)                    | Hospital acquisition                       | BRL 700 million   | Bloomberg / UOL                      |
| 2016 | UnitedHealth Group (via Amil) | United States | Hospital de Clínicas de Jacarepaguá (Rio de Janeiro) | Hospital acquisition                       | Undisclosed       | UnitedHealth Group corporate reports |
| 2016 | UnitedHealth Group (via Amil) | United States | Casa de Saúde Santa Therezinha (Rio de Janeiro)      | Hospital acquisition                       | Undisclosed       | UnitedHealth Group corporate reports |
| 2016 | UnitedHealth Group (via Amil) | United States | Grupo Ana Costa (São Paulo)                          | Hospital group acquisition                 | Undisclosed       | <i>Valor Econômico</i>               |
| 2018 | UnitedHealth Group (via Amil) | United States | Grupo Sobam (Hospital Pitangueiras)                  | Hospital group acquisition                 | Undisclosed       | Amil statement / SEGS                |
| 2025 | CVC Capital Partners          | Luxembourg    | GSH Corp.                                            | Acquisition of a hospital services company | BRL 1.5 billion   | CVC statement                        |

Source: Prepared by the authors based on *Folha de S.Paulo*, Reuters, UOL, Bloomberg, UnitedHealth Group corporate reports, *Valor Econômico*, Amil, SEGS, and CVC Capital Partners (2026).

The temporal distribution of the catalogued transactions is not uniform. Seven of the nine transactions (78%) were concentrated in 2015 and 2016, immediately after the enactment of Federal Law No. 13,097/2015, which authorized unrestricted foreign investment in the healthcare sector (Scheffer, 2022). This concentration is apparently supported Dunning's (2001) Eclectic Paradigm, according to which the attractiveness of a market to foreign capital depends directly on its institutional and regulatory conditions. The rapid response of investors suggests the existence of previously constrained demand that had been limited by the former legal restrictions. After this initial peak, no new transactions were identified between 2019 and 2024, and only one operation was recorded in 2025. This pattern may reflect either limitations in the available media coverage or a genuine slowdown in foreign investment.

It is important to note that not all catalogued transactions involved direct hospital acquisitions. Of the nine operations, only five consisted of the direct acquisition of a hospital or hospital group by a foreign investor, all involving UnitedHealth Group through Amil. The remaining transactions comprised equity stake acquisitions in established hospital networks, such as GIC's and The Carlyle Group's investments in Rede D'Or, or the acquisition of a complementary healthcare services company, as in CVC Capital Partners' purchase of GSH Corp., a hemotherapy specialist. This distinction reflects the difference between high- and low-commitment entry modes discussed by Cavusgil, Knight, and Riesenberger (2010) and Kotabe and Helsen (2017). Full hospital acquisitions imply complete operational control, whereas equity investments involve a lower level of commitment and are more characteristic of financial investors seeking returns without directly managing healthcare operations.

This distinction is also evident in the investor profile. Six of the nine transactions (67%) involved UnitedHealth Group through Amil. The company's presence in Brazil since 2012 provided a platform for subsequent acquisitions with lower transaction costs, illustrating a gradual process of operational internationalization. By contrast, GIC, The Carlyle Group, and CVC Capital Partners represent financial investors whose primary interest lies in acquiring equity stakes or specific assets within the healthcare ecosystem rather than directly operating hospitals.

Regarding the geographical origin of investors, entities from the United States accounted for seven of the nine transactions, followed by Singapore and Luxembourg. This predominance is consistent with the argument advanced by Ouakil, Sarsar, and Soussane (2025), who emphasize the attractiveness of healthcare markets characterized by strong demand and evolving institutional infrastructure. The Brazilian case, however, is distinguished by the coexistence of a universal public healthcare system (SUS) and an expanding private healthcare sector (Paim, Travassos, Almeida, Bahia, & Macinko, 2011).

Finally, four of the nine transactions (44%), all involving UnitedHealth Group through Amil, had undisclosed financial values. This limitation likely underestimates the total volume of foreign capital invested during the period. Nevertheless, the evidence indicates that the internationalization of the Brazilian hospital sector between 2015 and 2025 combined two complementary dynamics.

The first was the strategic consolidation of a single investor through direct acquisitions that expanded operational control. The second was the selective entry of international financial capital through equity investments in established hospital networks and related healthcare assets. These findings are consistent with Burns, Wholey, and Kralovec (2015) and Postma and Roos (2016), who argue that mergers and acquisitions simultaneously serve as mechanisms for entering regulated markets and as strategies for improving operational efficiency and strengthening competitive positioning.

## 5 CONCLUSION

This study mapped foreign acquisitions in the Brazilian hospital sector between 2015 and 2025 following the regulatory liberalization introduced by Federal Law No. 13,097/2015. The findings show that foreign investment was concentrated in the two years immediately after the reform and was driven primarily by UnitedHealth Group, through Amil, which pursued a strategy of direct hospital acquisitions. In contrast, GIC, The Carlyle Group, and CVC Capital Partners entered the sector mainly through equity investments or complementary healthcare assets.

The results are consistent with Dunning's (2001) Eclectic Paradigm, highlighting the importance of institutional openness and regulatory stability in attracting foreign direct investment. However, the concentration of transactions in a single investor and the limited disclosure of transaction values suggest that the available evidence captures only part of the broader process of foreign capital participation in Brazilian healthcare. Overall, this study provides an initial empirical overview of the patterns and characteristics of foreign acquisitions in the Brazilian hospital sector during the first decade following regulatory liberalization.

A central limitation of this study concerns access to reliable information on hospital acquisitions by foreign investors in Brazil. Public sources capture only part of these transactions, since many of the counterparts involved are privately held companies not subject to mandatory financial disclosure; the dataset compiled here should therefore be read as an approximation rather than an exhaustive record.

## REFERENCES

- BLANCO, Ana C. Amil, de UnitedHealth Group, adquire Hospital Santa Helena. LexLatin, 31 out. 2016. Disponível em: <https://lexlatin.com/noticias/amil-de-unitedhealth-group-adquire-hospital-santa-helena>. Acesso em: 8 jul. 2026.
- BRAZIL. Ministry of Health. Brasília, 2026.
- BUENO, Denise. Amil anuncia a aquisição do Grupo Sobam. Sonho Seguro, 11 set. 2018. Disponível em: <https://www.sonhoseguro.com.br/2018/09/amil-anuncia-a-aquisicao-do-grupo-sobam/>. Accessed em: 8 jul. 2026.
- BURNS, Lawton R.; MCCULLOUGH, Jeffrey S.; WHOLEY, Douglas R.; KRUSE, Gregory; KRALOVEC, Peter; MULLER, Ralph. Is the system really the solution? Operating costs in hospital systems. Medical Care Research and Review, v. 72, n. 3, p. 247–272, 2015.
- CARLYLE compra 8,3% da Rede D'Or São Luiz por R\$ 1,75 bilhão. Bloomberg/UOL, São Paulo, abr. 2015. Disponível em: <https://www.uol.com.br/>. Accessed in: 01 ago 2026.
- CAVUSGIL, S. Tamer; KNIGHT, Gary; RIESENBERGER, John R. International business: the new realities. 2nd ed. Upper Saddle River: Pearson Prentice Hall, 2010.
- CAVUSGIL, S. Tamer; KNIGHT, Gary; RIESENBERGER, John R. Negócios internacionais: estratégia, gestão e novas realidades. São Paulo: Pearson Prentice Hall, 2010.
- CVC Capital Partners acquires Brazilian healthcare firm GSH in R\$1.58 billion strategic move. The Rio Times, Rio de Janeiro, 17 abr. 2025. Disponível em: <https://www.riotimesonline.com/cvc-capital-partners-acquires-brazilian-healthcare-firm-gsh-in-r1-58-billion-strategic-move/>. Acesso em: 22 jul. 2026.
- DUNNING, J. H. The Eclectic (OLI) Paradigm of International Production: Past, Present and Future. International Journal of the Economics of Business, v. 8, n. 2, p. 173–190, 2001.
- FREITAS, Tatiana. Dona da Amil confirma compra do Hospital Samaritano em SP. Folha de S.Paulo, São Paulo, 11 dez. 2015. Disponível em: <https://www1.folha.uol.com.br/mercado/2015/12/1717978-hospital-samaritano-confirma-ven-da-para-dona-da-amil.shtml>. Acesso em: 1 ago. 2026.
- FUNDO soberano de Cingapura (GIC) compra 16% da Rede D'Or São Luiz. Reuters/UOL, São Paulo, 27 maio 2015. Disponível em: <https://economia.uol.com.br/noticias/reuters/2015/05/27/fundo-soberano-de-cingapura-compra-fatia-da-rede-dor-por-r33-bi-dizem-fontes.htm>. Accessed in: 01 ago. 2026.
- JOHANSON, J.; VAHLNE, J.-E. The Uppsala internationalization process model revisited: From liability of foreignness to liability of outsidership. Journal of International Business

Studies, v. 40, n. 9, p. 1411–1431, 2009.

KAMIA, Felipe Duvaresch; VARGAS, Marco Antonio. Capital estrangeiro e mudança estrutural no mercado privado de serviços de saúde brasileiro. Cadernos de Saúde Pública, Rio de Janeiro, v. 39, suppl. 2, e00171222, 2023.

KOIKE, Beth. Dona da Amil compra Hospital Samaritano, em SP. Folha de S.Paulo, São Paulo, 30 nov. 2015. Disponível em: <https://www1.folha.uol.com.br/>. Accessed in: 12 ago. 2026.

KOTABE, Masaaki; HELSEN, Kristiaan. Global marketing management. 7th ed. Hoboken: John Wiley & Sons, 2017.

LUCCHESI, Cristiane; LEVIN, Jonathan. UnitedHealth negocia aquisição do Hospital Santa Helena, segundo fontes. UOL Economia, São Paulo, 13 jan. 2016. Disponível em: <https://economia.uol.com.br/noticias/bloomberg/2016/01/13/unitedhealth-negocia-aquisicao-do-hospital-santa-helena-por-r-700-milhoes-fontes.htm>. Acesso em: 10 jul. 2026.

OECD. OECD Reviews of Health Systems: Brazil 2021. Paris: OECD Publishing, 2021. Available at: <https://doi.org/10.1787/146d0dea-en>. Accessed: July 17, 2026.

ORGANISATION FOR ECONOMIC CO-OPERATION AND DEVELOPMENT (OECD). OECD. Paris. Available at: <https://www.oecd.org/>. Accessed: July 21, 2026.

OUAKIL, Hicham; SARSAR, Hana; AIT SOUSSANE, Jihad. Does universalization of healthcare coverage promote inward FDI in health sectors? An empirical investigation using panel data of 59 countries. Globalization and Health, v. 21, art. 18, 2025.

PAIM, J. S.; TRAVASSOS, C. M. D. R.; ALMEIDA, C. M. D.; BAHIA, L.; MACINKO, J. O sistema de saúde brasileiro: história, avanços e desafios. 2011.

PEDUZZI, M. O SUS é interprofissional. Interface — Comunicação Saúde Educação, v. 20, n. 56, 2016.

POSTMA, J.; ROOS, A. F. Why healthcare providers merge. Health Economics, Policy and Law, v. 11, n. 2, p. 121–135, 2016.

SALOMÃO, Karin. UnitedHealth, dona da Amil, compra grupo Ana Costa. Exame, São Paulo, 16 dez. 2016. Disponível em: <https://exame.com/negocios/unitedhealth-dona-da-amil-compra-grupo-ana-costa/>. Acesso em: 8 jul. 2026.

SCHEFFER, Mário; SOUZA, Paulo Marcos Senra. A entrada do capital estrangeiro no sistema de saúde no Brasil. Cadernos de Saúde Pública, Rio de Janeiro, v. 38, suppl. 2, e00239421, 2022.

SEGS. Amil anuncia a aquisição do Grupo Sobam. SEGS, São Paulo, 11 set. 2018. Disponível em: <https://www.segs.com.br/seguuros/134505-amil-anuncia-a-aquisicao-do-grupo-sobam>. Acesso em: Accessed em: 8 jul. 2026.

SHENKAR, Oded; LIANG, Guoyong; SHENKAR, Rakefet. The last frontier of globalization: Trade and foreign direct investment in healthcare. Thunderbird International Business Review, v. 63, n. 4, p. 509–520, 2021.

VALOR ECONÔMICO. UnitedHealth, dona da Amil, compra Grupo Ana Costa, de Santos. Valor Econômico, São Paulo, 16 dez. 2016. Disponível em: <https://valor.globo.com/>. Accessed in: 29 jul. 2026.

VALOR ECONÔMICO. UnitedHealth, dona da Amil, compra Grupo Ana Costa, de Santos. Valor Econômico, São Paulo, 16 dez. 2016. Disponível em: <https://valor.globo.com/>. Accessed in: 01 ago. 2026.

WORLD BANK. World Bank. Washington, DC, 2026. Available at: <https://www.worldbank.org/>. Accessed: July 13, 2026.

WORLD HEALTH ORGANIZATION (WHO). World Health Organization. Geneva, 2026. Available at: <https://www.who.int/>. Accessed: July 21, 2026.

#### **AI USE STATEMENT:**

During the preparation of this work, the authors used ChatGPT for the purpose of correcting it according to ABNT standards. After its use, the content was reviewed and edited by the authors themselves, who assume full responsibility for all information presented in the published article.